# Urology - Operative Notes

## Record 1

ADMITTING DIAGNOSES:,1. Hematuria.,2. Benign prostatic hyperplasia.,3. Osteoarthritis.,DISCHARGE DIAGNOSES:,1. Hematuria, resolved.,2. Benign prostatic hyperplasia.,3. Complex renal cyst versus renal cell carcinoma or other tumor.,4. Osteoarthritis.,HOSPITAL COURSE:, This is a 77-year-old African-American male who was previously well until he began having gross hematuria and clots passing through his urethra on the day of admission. He stated that he never had blood in his urine before, however, he does have a past history of BPH and he had a transurethral resection of prostate more than 10 years ago. He was admitted to a regular bed. Dr. G of Urology was consulted for evaluation of his hematuria. During the workup for this, he had a CT of the abdomen and pelvis with and without contrast with early and late-phase imaging for evaluation of the kidneys and collecting system. At that time, he was shown to have multiple bilateral renal cysts with one that did not meet classification as a simple cyst and ultrasound was recommended.,He had an ultrasound done of the cyst which showed a 2.1 x 2.7 cm mass arising from the right kidney which, again, did not fit ultrasound criteria for a simple cyst and they recommended further evaluation by an MRI as this could be a hemorrhagic cyst or a solid mass or tumor, so an MRI was scheduled on the day of discharge for further evaluation of this. The report was not back at discharge. The patient had a cystoscopy and transurethral resection of prostate as well with entire resection of the prostate gland. Pathology on this specimen showed multiple portions of prostatic tissue which was primarily fibromuscular, and he was diagnosed with nonprostatic hyperplasia. His urine slowly cleared. He tolerated a regular diet with no difficulties in his activities of daily living, and his Foley was removed on the day of discharge.,He was started on ciprofloxacin, Colace, and Lasix after the transurethral resection and continued these for a short course. He is asked to continue the Colace as an outpatient for stool softening for comfort.,DISCHARGE MEDICATIONS:, Colace 100 mg 1 b.i.d.,DISCHARGE FOLLOWUP PLANNING:, The patient is to follow up with his primary care physician at ABCD, Dr. B or Dr. J, the patient is unsure of which, in the next couple weeks. He is to follow up with Dr. G of Urology in the next week by phone in regards to the patient's MRI and plans for a laparoscopic partial renal resection biopsy. This is scheduled for the week after discharge potentially by Dr. G, and the patient will discuss the exact time later this week. The patient is to return to the emergency room or to our clinic if he has worsening hematuria again or no urine output.

## Record 2

CIRCUMCISION - OLDER PERSON,OPERATIVE NOTE:, The patient was taken to the operating room and placed in the supine position on the operating table. General endotracheal anesthesia was administered. The patient was prepped and draped in the usual sterile fashion. A 4-0 silk suture is used as a stay-stitch of the glans penis. Next, incision line was marked circumferentially on the outer skin 3 mm below the corona. The incision was then carried through the skin and subcutaneous tissues down to within a layer of * fascia. Next, the foreskin was retracted. Another circumferential incision was made 3 mm proximal to the corona. The intervening foreskin was excised. Meticulous hemostasis was obtained with electrocautery. Next, the skin was reapproximated at the frenulum with a U stitch of 5-0 chromic followed by stitches at 12, 3, and 9 o'clock. The stitches were placed equal distance among these to reapproximate all the skin edges. Next, good cosmetic result was noted with no bleeding at the end of the procedure. Vaseline gauze, Telfa, and Elastoplast dressing was applied. The stay-stitch was removed and pressure held until bleeding stopped. The patient tolerated the procedure well and was returned to the recovery room in stable condition.

## Record 3

DIAGNOSIS: , Pubic cellulitis.,HISTORY OF PRESENT ILLNESS:, A 16-month-old with history of penile swelling for 4 days. The patient was transferred for higher level of care. This 16-month-old had circumcision 1 week ago and this is the third circumcision this patient underwent. Apparently, the patient developed adhesions and the patient had surgery for 2 more occasions for removal of the adhesions. This time, the patient developed fevers 3 days after the surgery with edema and erythema around the circumcision and it has spread to the pubic area. The patient became febrile with 102 to 103 fever, treated with Tylenol with Codeine and topical antibiotics. The patient was transferred to Children's Hospital for higher level of care.,REVIEW OF SYSTEMS: , ,ENT: Denies any runny nose. ,EYES: No apparent discharge. ,FEEDING: Good feeding. ,CARDIOVASCULAR: There is no cyanosis or edema. ,RESPIRATORY: Denies any cough or wheezing. ,GI: Positive for constipation, no bowel movements for 2 days. ,GU: Positive dysuria for the last 2 days and penile discharge for the last 2 days with foul smelling. ,NEUROLOGIC: Denies any lethargy or seizure. ,MUSCULOSKELETAL: No pain or swelling. ,SKIN: Erythema and edema in the pubic area for the last 3 days. All the rest of systems are negative except as noted above.,At the emergency room, the patient had a second dose of clindamycin. The transfer labs are as follows: 15.7 for WBC, H&H 12.0 and 36. One blood culture. We will follow the results. He is status post Rocephin and Cleocin.,PAST MEDICAL HISTORY: , Denied. ,PAST SURGICAL HISTORY:, The patient underwent 3 circumcisions since birth, the last 2 had been for possible removal of adhesions.,IMMUNIZATIONS: , He is behind with his immunizations. He is due for his 16-month-old immunizations.,ACTIVITY: , NKDA.,BIRTH HISTORY: , Born to a 21-year-old, first baby, born NSVD, 8 pounds 10 ounces, no complications.,DEVELOPMENTAL:, He is walking and speaking about 15 words.,FAMILY HISTORY: , Noncontributory.,MEDICATIONS: , Tylenol with Codeine q.6h.,SOCIAL HISTORY: , He lives with both parents and both of them smoke. There are no pets.,SICK CONTACTS: , Mom has some upper respiratory infection.,DIET: , Regular diet.,PHYSICAL EXAMINATION: , ,VITAL SIGNS: Temperature max at ER is 102, heart rate 153.,GENERAL: This patient is alert, arousable, big boy.,HEENT: Head: Normocephalic, atraumatic. Pupils are equal, round, and reactive to light. Mucous membranes are moist.,NECK: Supple.,CHEST: Clear to auscultation bilaterally. Good air exchange.,ABDOMEN: Soft, nontender, nondistended.,EXTREMITIES: Full range of movement. No deformities.

## Record 4

REASON FOR VISIT: , Followup circumcision.,HISTORY OF PRESENT ILLNESS: , The patient had his circumcision performed on 09/16/2007 here at Children's Hospital. The patient had a pretty significant phimosis and his operative course was smooth. He did have a little bit of bleeding when he woke in recovery room, which required placement of some additional sutures, but after that, his recovery has been complete. His mom did note that she had to him a couple of days of oral analgesics, but he seems to be back to normal and pain free now. He is having no difficulty urinating, and his bowel function remains normal.,PHYSICAL EXAMINATION: ,Today, The patient looks healthy and happy. We examined his circumcision site. His Monocryl sutures are still in place. The healing is excellent, and there is only a mild amount of residual postoperative swelling. There was one area where he had some recurrent adhesions at the coronal sulcus, and I gently lysed this today and applied antibiotic ointment showing this to mom had to especially lubricate this area until the healing is completed.,IMPRESSION: , Satisfactory course after circumcision for severe phimosis with no perioperative complications.,PLAN: ,The patient came in followup for his routine care with Dr. X, but should not need any further routine surgical followup unless he develops any type of difficulty with this surgical wound. If that does occur, we will be happy to see him back at any time.,

## Record 5

PREOPERATIVE DIAGNOSIS:, Phimosis.,POSTOPERATIVE DIAGNOSES:, Phimosis.,OPERATIONS:, Circumcision.,ANESTHESIA: , LMA.,EBL:, Minimal.,FLUIDS: , Crystalloid.,BRIEF HISTORY: , This is a 3-year-old male, who was referred to us from Dr. X's office with phimosis. The patient had spraying of urine and ballooning of the foreskin with voiding. The urine seemed to have collected underneath the foreskin and then would slowly drip out. Options such as dorsal slit, circumcision, watchful waiting by gently pulling the foreskin back were discussed. Risk of anesthesia, bleeding, infection, pain, scarring, and expected complications were discussed. The patient's family understood all the complications and wanted to proceed with the procedure. Consent was obtained using interpreter.,DESCRIPTION OF PROCEDURE: ,The patient was brought to the OR and anesthesia was applied. The patient was placed in supine position. The patient was prepped and draped in usual sterile fashion. All the penile adhesions were released prior to the prepping. The extra foreskin was marked off, 1 x 3 Gamco clamp was used. Hemostasis was obtained after removing the extra foreskin using the Gamco clamp.,Using 5-0 Monocryl, 4 quadrant stitches were placed and horizontal mattress suturing was done. There was excellent hemostasis. Dermabond was applied. The patient was brought to recovery at the end of the procedure in stable condition.

## Record 6

PREOPERATIVE DIAGNOSIS:, Prostate cancer.,POSTOPERATIVE DIAGNOSIS:, Prostate cancer.,OPERATIONS: , Brachytherapy, iodine-125 seed implantation, and cystoscopy.,ANESTHESIA:, LMA.,ESTIMATED BLOOD LOSS: , Minimal.,Total number of seeds placed, 63. Needles, 24.,BRIEF HISTORY OF THE PATIENT: , This is a 57-year-old male who was seen by us for elevated PSA. The patient had a prostate biopsy with T2b disease, Gleason 6. Options such as watchful waiting, robotic prostatectomy, seed implantation with and without radiation were discussed. Risks of anesthesia, bleeding, infection, pain, MI, DVT, PE, incontinence, rectal dysfunction, voiding issues, burning pain, unexpected complications such as fistula, rectal injury, urgency, frequency, bladder issues, need for chronic Foley for six months, etc., were discussed. The patient understood all the risks, benefits, and options, and wanted to proceed with the procedure. The patient was told that there could be other unexpected complications. The patient has history of urethral stricture. The patient was told about the risk of worsening of the stricture with radiation. Consent was obtained.,DETAILS OF THE OPERATION: , The patient was brought to the OR. Anesthesia was applied. The patient was placed in the dorsal lithotomy position. The patient had SCDs on. The patient was given preop antibiotics. The patient had done bowel prep the day before. Transrectal ultrasound was performed. The prostate was measured at about 32 gm. The images were transmitted to the computer system for radiation oncologist to determine the dosing etc. Based on the computer analysis, the grid was placed. Careful attention was drawn to keep the grid away from the patient. There was a centimeter distance between the skin and the grid. Under ultrasound guidance, the needles were placed, first in the periphery of the prostate, a total of 63 seeds were placed throughout the prostate. A total of 24 needles was used. Careful attention was drawn to stay away from the urethra. Under longitudinal ultrasound guidance, all the seeds were placed. There were no seeds visualized in the bladder under ultrasound. There was only one needle where the seeds kind of dragged as the needle was coming out on the left side and were dropped out of position. Other than that, all the seeds were very well distributed throughout the prostate under fluoroscopy. Please note that the Foley catheter was in place throughout the procedure. Prior to the seed placement, the Foley was attempted to be placed, but we had to do it using a Glidewire to get the Foley in and we used a Councill-tip catheter. The patient has had history of bulbar urethral stricture. Pictures were taken of the strictures in the pre-seed placement cysto time frame. We needed to do the cystoscopy and Glidewire to be able to get the Foley catheter in. At the end of the procedure, again cystoscopy was done, the entire bladder was visualized. The stricture was wide open. The prostate was slightly enlarged. The bladder appeared normal. There was no sheath inside the urethra or in the bladder. The cysto was done using 30-degree and 70-degree lens. At the end of the procedure, a Glidewire was placed, and 18 Councill-tip catheter was placed. The plan was for Foley to be left in place overnight since the patient has history of urethral strictures. The patient is to follow up tomorrow to have the Foley removed. The patient could also be shown to have it removed at home.,The patient was brought to Recovery in stable condition at the end of the procedure. The patient tolerated the procedure well.